

Microbial Changes Upon Chemotherapy Induced GI Toxicity in GI Cancer Patients—a Population Study

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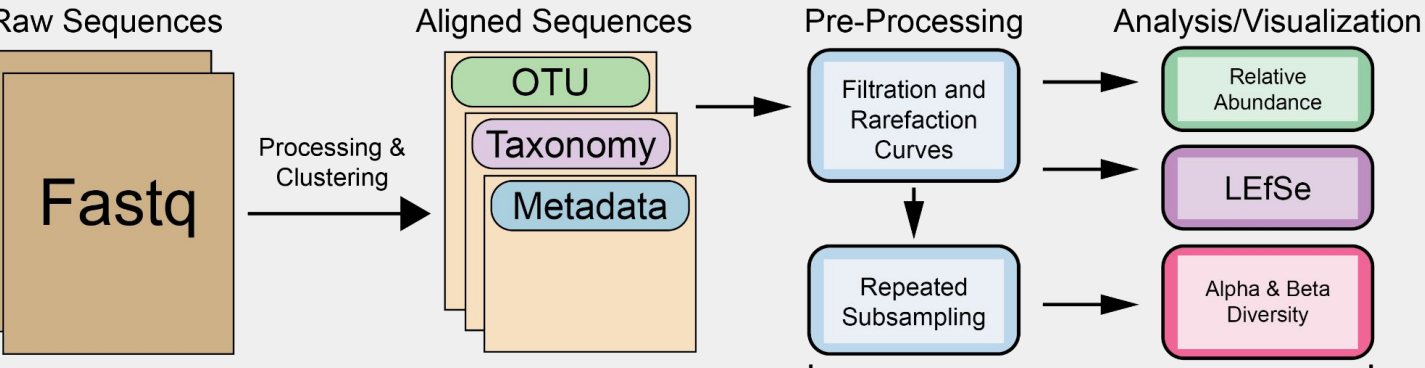
BACKGROUND

- Microbial drug metabolism can modulate chemotherapy efficacy and toxicity, linking microbiome composition to treatment outcomes. [1,2]
- Irinotecan, a broad-spectrum anticancer agent, disrupts the gut microbiome, where bacterial β -glucuronidase reactivates its metabolites (SN38G $\rightarrow$ SN38), driving diarrhea and mucositis. [3]
- Mild to severe GI toxicity occurs in 50–80% of patients treated with irinotecan, often leading to dose delays, reductions, or hospitalization. [4]

STUDY OVERVIEW

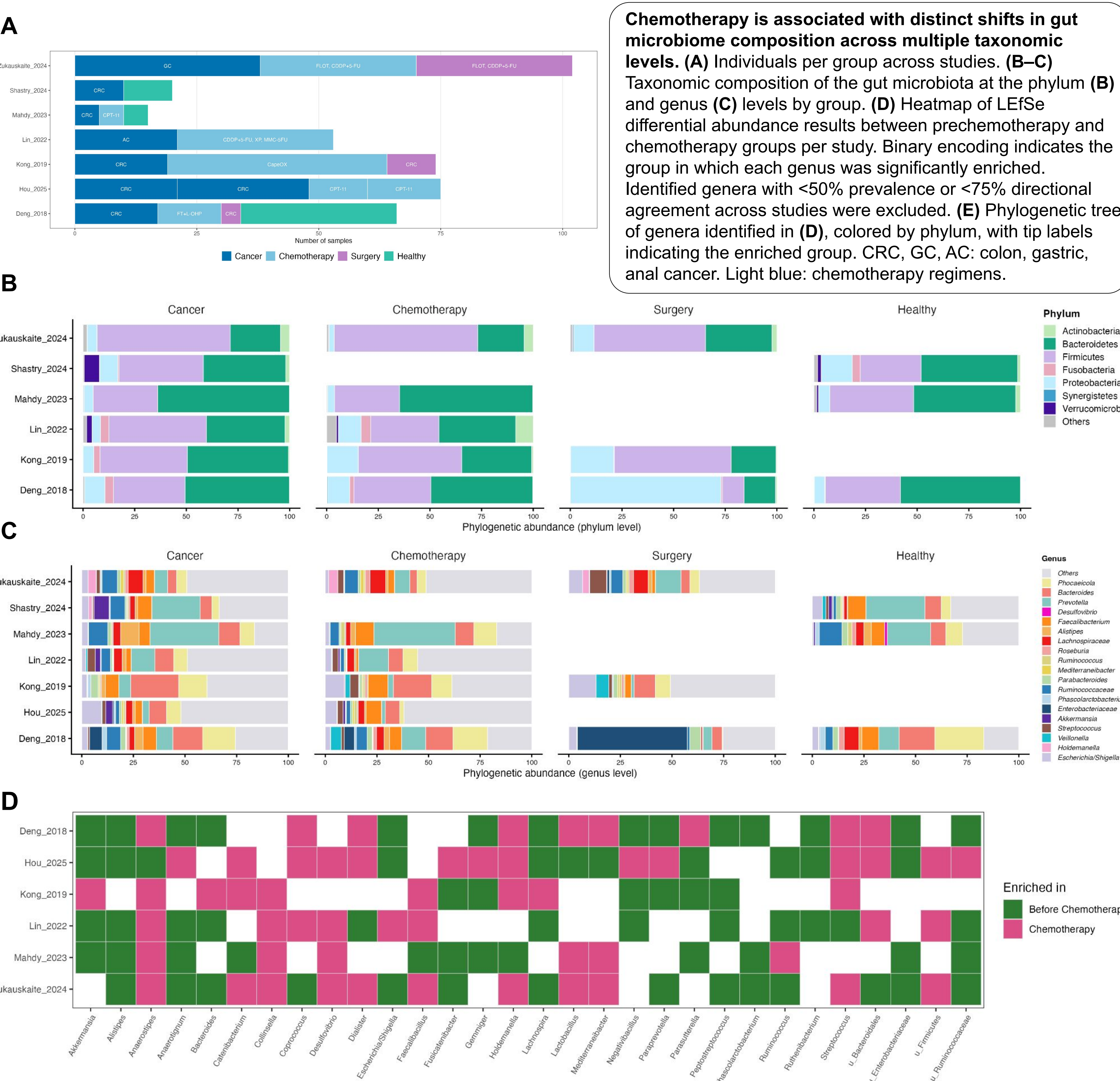
- Approach:** We harmonize existing clinical microbiome datasets to compare microbiome changes upon treatment with irinotecan and other chemotherapy regimens.
- Clinical data:** GI cancer patients grouped by condition (e.g., healthy, chemotherapy).
- Goal:** Identify microbial signatures consistently associated with chemotherapy-induced GI toxicity from clinical data to guide mechanistic and lab-based translational studies.

METHODS



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RESULTS

- We observe microbial shifts associated with chemotherapy-induced GI toxicity in GI cancer studies.
- Desulfovibrio (DSV), a sulfate-reducing, β -glucuronidase-producing bacterium, emerged as a taxon consistently associated with the chemotherapy treatment group across human studies.
- These findings suggest that hydrogen sulfide produced by DSV may be a significant proinflammatory molecule for chemotherapy-induced GI toxicity.
- Collinsella, a gut bacterium associated with epithelial barrier dysfunction and proinflammatory signaling, was also enriched in chemotherapy-treated groups.

NEXT STEPS

- Apply our harmonized pipeline to more studies to build a comprehensive chemotherapy-microbiome map.

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